



Retatrutide

Information sheet written for the 20 mg vial TrueHold holds in stock. Not a generic internet protocol.

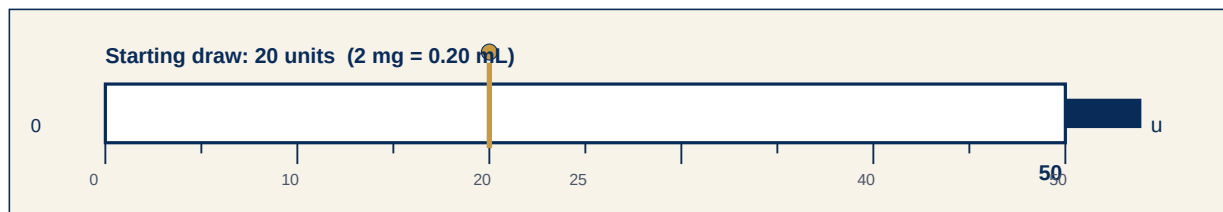
THIS VIAL — STARTING DOSE (read this first)

Retatrutide · 20 mg lyophilized · mix with 2 mL bacteriostatic water · 10 mg/mL · 1 unit = 100 mcg

Start: draw **20 units** on a U-100 syringe (**0.20 mL = 2 mg**). On a 0.5 mL (50-unit) U-100 syringe, draw to the 20 mark.

Route: Subcutaneous (belly or back of upper arm)

Rhythm: Once weekly. Stay on each level at least 4 weeks before going up.



1. What it is — the molecule, how large it is

Retatrutide (LY3437943) is a 39-amino-acid GIP-backbone peptide with a fatty-diacid albumin binder. Molecular weight about 4,731 daltons. It is a triple agonist: GLP-1 + GIP + glucagon receptors. TrueHold stocks a 20 mg lyophilized research vial — the same milligram size as our tirzepatide vial.

How incretin shots talk to the body

GLP-1 receptor

Appetite down
Stomach slower
Insulin (if glucose high)

GIP receptor

Insulin support
Fat-cell signaling

Glucagon receptor

Energy burn
(retatrutide only)

How large is it? (daltons, log-ish bars — bigger bar = bigger molecule)

GHK-Cu	404 Da
NAD+	663 Da
SS-31	749 Da
Semax	814 Da
MOTS-c	2175 Da
Retatrutide	4731 Da
Tirzepatide	4814 Da

2. How it works in the body

GLP-1 and GIP quiet appetite and improve insulin-response biology, as with tirzepatide. The extra glucagon-receptor arm is why researchers talk about higher energy expenditure and fat oxidation, not only eating less. Half-life is about six days, so it is a weekly subcutaneous shot. It is not AOD-9604 (a growth-hormone fragment) and not semaglutide.



3. Testing data — animal

Preclinical incretin/glucagon co-agonist work showed greater weight loss than dual agonists in obese animals, with the glucagon arm raising energy expenditure. Class thyroid C-cell findings in rodents still apply to anything that strongly stimulates GLP-1 receptors.

4. Testing data — human, what is expected, hopes

Phase 2 obesity work (NEJM 2023) and Phase 3 TRIUMPH-1 topline (Lilly, 2026) reported very large mean weight loss at 9–12 mg weekly, with GI events similar to other incretins plus a dysesthesia (odd skin sensation) signal that was uncommon on older GLP-1 drugs. Lilly has described filing plans; as of this sheet it is not approved. Trials start at 2 mg weekly and step every 4 weeks — that is why our 20 mg vial starts at 20 units (2.0 mg).

- Strong appetite control
- Weight-loss magnitude in trials that exceeded earlier incretins at high dose
- Possible extra energy-expenditure from the glucagon receptor
- Weekly schedule

5. Have people hurt themselves? Risks

Trial participants have had substantial GI illness: nausea, vomiting, diarrhea, constipation — more often at 9–12 mg. Dysesthesia (tingling, burning, odd skin feeling) showed up in Phase 3 at higher rates than placebo. Expect the same rare serious class risks as tirzepatide (pancreatitis, gallbladder, dehydration, aspiration under anesthesia) because GLP-1 is still on. There is no long-term post-marketing safety database yet. This is not a beginner 'more is better' peptide.

6. FDA — approved or not

NOT FDA-APPROVED. Investigational Lilly molecule. No legal retail drug product. Any seller claiming to ship 'pharmaceutical retatrutide pens' is not selling an approved medicine. TrueHold's 20 mg vial is a research formula, not a licensed drug.

7. Reconstitution for this exact vial

Check the label: it must say Retatrutide and 20 mg powder in a standard ~3 mL glass vial. You need bacteriostatic water (BAC), alcohol swabs, and U-100 insulin syringes (0.5 mL / 50-unit for every starting draw on this sheet; 1 mL / 100-unit if a later step exceeds 50 units).

- Wash hands. Wipe the rubber top with an alcohol swab.
- Draw **2 mL** of BAC into a mixing syringe.
- Insert through the stopper. Inject slowly down the inside glass wall — not straight onto the powder.
- Gently roll or swirl until clear. Do not shake.
- Wipe the top again. Write today's date on the vial. Refrigerate 36–46°F. Use within 28 days.
- Mix and store in this vial only. Do not transfer.

8. Loading the 0.5 mL syringe — schedule for this vial

U-100 math is the same on a 0.5 mL or 1 mL syringe: 100 units = 1.00 mL, so 50 units = 0.50 mL (a completely full 0.5 mL syringe). Starting draws on this sheet all fit 0.5 mL. The green row is the start.

When	Units (U-100)	mg	mL	0.5 mL syringe	How often
Weeks 1–4 (start here)	20	2	0.20	Draw to 20	Once per week
Weeks 5–8 (if tolerated)	40	4	0.40	Draw to 40	Once per week
Weeks 9–12 (optional)	60	6	0.60	Too big — 1 mL syringe or 50+ remainder	Once per week
Week 13+ (optional, trial-range)	90	9	0.90	Too big — 1 mL syringe or 50+ remainder	Once per week

- Use 2 mL bacteriostatic water only — this vial cannot hold more.
- 20 units = 2.0 mg once weekly, matching the TRIUMPH Phase 3 start (2 mg, then step up every 4 weeks).
- Lilly trial maintenance doses go to 9–12 mg. 12 mg would be 120 units (two syringes) and is not a TrueHold start.



- At 20 units/week one mixed vial lasts 10 weekly doses.

Fulfillment

- Prep and local delivery: Las Vegas residents only.
- Shipping: dry (lyophilized) vials only — we do not ship mixed liquid.
- Interest orders are confirmed by phone before payment.

Working with the team

- Share a phone number so the team can call.
- Confirm Las Vegas residency for prep and local delivery.
- Complete required documentation with the team.
- If anything on this sheet disagrees with your vial label or COA, stop and call before mixing.

Sources

- Lilly TRIUMPH-1 Phase 3 obesity topline: <https://investor.lilly.com/node/54321/pdf>
- InvivoGen / public chemistry sheets: ~4731 Da, 39 amino-acid backbone
- Drugs.com retatrutide status: not FDA-approved
- Same 20 mg / 2 mL mix as TrueHold tirzepatide. Milligram steps follow published TRIUMPH start (2 mg weekly), not an invented scale.

Protocol note

TrueHold writes these sheets for the milligram vials we hold. Mix volume and unit marks were double-checked so milligrams, milliliters, and U-100 units agree. This is educational research information, not a prescription and not medical advice. The Telegram bot does not dose in chat — read this PDF. For an individualized plan, use /schedule. Research use only.

